

JOHNS HOPKINS MEDICINE
NEUROLOGY REFERRAL INTAKE (FAX RECEIVED)

Date: 03/14/2026 11:42 AM

Fax ID: 443-XXX-8821

Pages Received: 11 (incl cover)

Referral Reason: "episodic neurologic symptoms ?migraine vs vascular event"

Requested Specialty: General Neurology (provider unsure)

Urgency: Routine (but pt "concerned")

Referring Office: Fair Oaks Primary Care Assoc.

Comments:

Pt w/ intermittent R-sided weakness, resolved now. Possibly stress related. Family hx stroke.
Please evaluate.

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PATIENT DEMOGRAPHICS (auto-export)

Name: MARTINEZ, LUCAS J

DOB: 08/22/1972

Age: 53

Sex: M

MRN (external): FO-992144

SSN: xxx-xx-8831

Address: 14322 Cedar Run Rd, Centreville VA 20120

Phone: (571) 555-0912

Alt Phone: none listed

Insurance: CareFirst BCBS PPO

Subscriber ID: QXX998234

Group: 88021

Emergency Contact:

Maria Martinez (spouse) 571-555-8821

Preferred Language: English

Interpreter: No

Allergies: NKDA (note: older chart says "?penicillin rash childhood")

Primary Care: Dr. R. Halvorsen

Referring Provider: same

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REFERRING PROVIDER NOTE (03/12/2026)

Provider: R. Halvorsen MD

Chief Complaint: "right arm felt weak again yesterday"

HPI:

53 y/o male w/ hx HTN, "borderline lipids," GERD presents for follow up after episode yesterday afternoon approx 3:30 PM. Pt reports sudden onset heaviness/weakness in R arm and possibly leg (unclear). Lasted ~10–15 min, resolved spontaneously. No loss of consciousness. Denies facial droop but wife thought speech "slightly off" (pt disagrees).

Pt reports similar episode ~2 weeks ago but thought it was "pinched nerve" while working at computer.

Denies headache at time of episode though later developed mild dull headache (non-throbbing). No photophobia or nausea.

No visual aura per pt, but says "maybe some blurriness" prior to first episode (uncertain timing).

ROS:

Neuro: transient weakness, no persistent deficit

CV: no chest pain

Resp: neg

GI: GERD controlled

Psych: increased stress at work

PMH:

HTN

Hyperlipidemia (not on statin currently)

GERD

"migraine headaches?" (pt unsure, not formally dx)

FH: father stroke age 62

Exam:

Vitals stable (BP 148/92)

Neuro exam normal at time of visit

Assessment:

Transient neurologic symptoms, unclear etiology

Consider TIA vs migraine variant vs anxiety

Plan:

Refer neurology

Encouraged ER if recurrence

Consider imaging if symptoms persist

Signed electronically

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PRIOR NOTE #1 (URGENT CARE VISIT)

Date: 02/27/2026

Location: Fairfax Walk-In Clinic

Chief Complaint: "arm numbness"

HPI:

Pt reports woke this morning w/ tingling R hand and forearm, lasted approx 20 min then resolved. Also felt "slightly weak." No facial symptoms. No speech changes.

Pt states has been having headaches intermittently for "years" though none today.

Denies trauma. Works long hours typing.

Exam:

Neuro grossly intact

Grip strength normal

No focal deficits

Assessment:

Likely peripheral nerve compression vs anxiety

Low suspicion for central process

Plan:

Rest, NSAIDs

Follow up PCP

Note:

Pt declined ER referral at that time

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PRIOR NOTE #2 (PRIMARY CARE ANNUAL, COPY-FORWARDED SECTIONS)

Date: 01/10/2026

Chief Complaint: Annual physical

HPI:

No acute complaints. Occasional headaches, described as "pressure," no aura, no nausea. Occurs monthly.

(copy forward begins)

PMH: HTN, GERD, hyperlipidemia

FH: father CVA, mother DM

SH: denies smoking (older record says “former smoker quit 2008”)

Medications:

Lisinopril 10 mg daily

Omeprazole 20 mg daily

Multivitamin

Exam normal

Assessment: stable chronic conditions

Plan: lifestyle modification

(copy forward ends)

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PRIOR NOTE #3 (ER VISIT – PARTIAL RECORD, TRUNCATED)

Facility: Inova Fair Oaks Hospital

Date: 03/01/2026

Chief Complaint: “weakness resolved”

HPI:

Patient presents approx 1 hr after episode of R-sided weakness and mild difficulty speaking per spouse. Symptoms lasted ~10 minutes and resolved prior to arrival.

Patient denies headache initially but later reports mild posterior headache (3/10).

No prior stroke history.

Exam:

NIHSS 0 at time of eval

CT Head: negative (see report below)

Disposition:

Patient left prior to full evaluation (“did not want to stay”)

ED physician recommended admission for observation and further workup.

Note truncated after discharge discussion

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LABS (mixed dates)

03/01/2026 (ER):

CBC: WNL

CMP: WNL

Glucose: 118

Creatinine: 1.0

Troponin: negative

Lipid panel (01/10/2026):

Total Chol: 228

LDL: 148

HDL: 38

Triglycerides: 190

HbA1c: 5.8%

TSH: 2.1

Vitamin D: 22 (low)

PSA: 1.2

Urinalysis: normal

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IMAGING REPORTS

CT HEAD WITHOUT CONTRAST

Date: 03/01/2026

Indication: transient neurologic deficit

Findings:

No acute intracranial hemorrhage.

No mass effect or midline shift.

Ventricles normal size.

Impression: No acute intracranial abnormality.

Carotid Ultrasound (ordered outpatient, 03/05/2026):

Right ICA: mild plaque, <50% stenosis

Left ICA: minimal plaque
No hemodynamically significant stenosis

MRI BRAIN: NOT COMPLETED (patient “unable to schedule yet”)

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MEDICATION LIST (CURRENT, AUTO-GENERATED)

Lisinopril 10 mg daily
Omeprazole 20 mg daily
Aspirin 81 mg daily (started by patient after ER visit, not prescribed)
Ibuprofen PRN
Vitamin D 1000 IU daily
Fish oil supplement
Melatonin PRN sleep
Albuterol inhaler (rare use, “for bronchitis years ago”)

Note: pt inconsistently takes meds per PCP note

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DISCHARGE SUMMARY (INCOMPLETE, ER VISIT)

Patient presented with transient right-sided weakness. Symptoms resolved spontaneously.

Workup initiated but incomplete due to patient leaving against medical advice.

Risks of leaving explained including possible stroke.

Advised to return if symptoms recur.

No medications prescribed at discharge.

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PATIENT INTAKE FORM (HANDWRITTEN, SCANNED)

Reason for visit:
“arm felt dead for a few minutes twice now”

Symptoms checklist:
☒ weakness
☒ numbness
☐ vision loss (scribbled “maybe blurry?”)
☐ headache (later added “afterwards”)

Duration: “10 min? maybe less”

Frequency: “2–3 times”

Additional comments:

“under a lot of stress at work, not sleeping great”

Signature: L. Martinez

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